

Women's Health Clinical Reference

Menopausal hormone therapy · Provider use only

KORB Health Medical Texas PA · Women's Health Clinical Reference v1.0 · effective 2026-09-16 · built 2026-09-16

Menopausal hormone therapy: estradiol, progesterone, testosterone and estriol. Pricing is tiered by how many hormones a plan contains, not by how many prescriptions are written, and the available strengths differ between the two partner pharmacies. Both of those are easy to get wrong and both are set out below.

Supersedes. The Women's Health Clinical Reference PDF of 2026-09-10, which was hand-produced and carried no version or effective date.

How the programme works

Four hormones can appear in a plan: estradiol, progesterone, testosterone and estriol. Each counts as ONE hormone toward the price, and one product can carry two - a progesterone and estradiol cream is a two-hormone plan in a single tube.

Prescriptions and dosing are quarterly - one visit, one 90-day supply.

Creams go to a partner pharmacy. Local pharmacies take the estradiol PATCH and commercial capsules only. NO topical creams and no compounded item goes to a local pharmacy - those go to Premier or Belmar.

The three stages

Menopause happens in three stages. PERIMENOPAUSE - fluctuating hormones and irregular periods. MENOPAUSE - twelve months without a period, with significant hormone decline. POSTMENOPAUSE - ongoing low hormone levels and rising risk of osteoporosis and heart disease.

Who is a candidate

Candidates typically have moderate to severe symptoms affecting quality of life.

- Hot flashes
- Night sweats
- Vaginal dryness or painful intercourse
- Mood disturbance
- Sleep problems
- Cognitive complaints - brain fog

Best outcomes when therapy starts under age 60 AND within 10 years of the last menstrual period. Lower cardiovascular risk inside that window of opportunity, which is the single most useful thing to establish at the first visit.

Women in early menopause are at higher risk of osteoporosis and heart disease, and usually BENEFIT from hormone therapy until at least the average age of natural menopause, around 51, unless contraindicated.

First question: does she still have a uterus?

This determines the whole plan, so establish it before anything else.

- YES - estrogen must ALWAYS be paired with progesterone. Unopposed estrogen causes endometrial hyperplasia and can lead to endometrial cancer. There is no dose of estrogen that is safe on its own here.
- NO - estradiol alone is first-line. Progesterone is not required, because there is no endometrium to protect. Adding it is not harmful but it is not indicated, and it moves the patient up a pricing tier for no benefit.

A patient with regular cycles takes progesterone 21 days on and 7 off, which is 63 capsules per 90 days. A post-menopausal patient takes it daily, which is 90. Getting this wrong means the quantity on the prescription does not match the regimen.

What each hormone is for, and when to use it

The clinical reason to choose each one, and what it changes about pharmacy and pricing.

Estradiol - patch or cream

FIRST-LINE for a patient WITHOUT a uterus, on its own.

What it does. Bioidentical estradiol through the skin for steady levels. Reduces hot flashes, night sweats and mood swings; supports vaginal health and sleep.

Why. Transdermal carries a lower clot risk than oral estrogen.

Pharmacy. Patch goes to Premier or the patient own pharmacy. Cream goes to a partner pharmacy only - never to a local pharmacy.

Unopposed estrogen in a patient WITH a uterus causes endometrial hyperplasia and can lead to cancer. Add progesterone.

Progesterone - capsule

REQUIRED alongside estrogen for any patient WITH a uterus.

What it does. Protects the endometrium. Also helps perimenopausal hot flashes, night sweats and mood, and taken at night it supports sleep.

Why. It is the reason estrogen is safe to give a woman who still has a uterus. This is not optional and it is not a preference.

Dosing. Start 200 mg nightly, increase to 300 mg if needed. Patients with regular cycles take it 21 days on, 7 days off - 63 capsules per 90 days. Patients without regular cycles take it daily - 90 capsules.

Pharmacy. 300 mg exists only as a compounded capsule. On a commercial prescription a 300 mg dose is one 200 mg plus one 100 mg.

PEANUT ALLERGY - do NOT use a commercial progesterone capsule, generic or brand. A COMPOUNDED capsule from Belmar or Premier is fine and is the answer here. The allergy rules out the product, not the hormone.

Testosterone - cream, compounded

Texas and California only. See the testosterone restriction.

What it does. Supports libido, energy, mood, muscle strength and cognition.

Dosing. Begin at one click daily of 2% cream - 5 mg per click - from a 30 mL dispenser, then adjust to response and side effects.

Pharmacy. Partner pharmacy only. Never to a local pharmacy.

Testosterone + estradiol cream

What it does. Both hormones in one application. Menopausal symptom relief plus sexual wellness, energy, mood and muscle tone.

Dosing. Testosterone 2% (1 click = 5 mg) with estradiol 0.4% (1 click = 1 mg). One click daily.

Billing. Two hormones for pricing, in a single tube.

Progesterone / testosterone / estradiol

What it does. Comprehensive support - vasomotor symptoms, mood, libido and energy.

Pharmacy. Testosterone and estradiol as cream, progesterone as capsules, from a partner pharmacy. Through a local pharmacy this becomes estradiol PATCH plus progesterone capsules only - no cream, no testosterone.

Billing. Three hormones for pricing.

Four-hormone oral capsule

What it does. Progesterone, testosterone, estriol and estradiol in one daily capsule, for a patient who would rather not use a topical.

Dosing. Progesterone 100 mg, testosterone 4 mg, estriol 0.45 mg, estradiol 0.45 mg. One daily, 90-day supply.

Billing. Bills at the three-hormone tier - four hormones, no fourth tier.

ORAL combination MAY INCREASE THE RISK OF BLOOD CLOTS. Transdermal does not carry the same risk, so this is a real trade-off to discuss rather than a formality.

Supply length and when to see her back

Book the follow-up at 12 WEEKS for every patient, whatever she is on. It is at or inside the shortest supply the programme writes, so nobody runs out waiting, and it is one number rather than two.

Product	Supply	Why
Estradiol patch	84 days	3 boxes of 4 = 12 patches, one a week
Progesterone, cycled	84 days	63 capsules, 21 on and 7 off, three cycles
Progesterone, daily	90 days	90 capsules, one a night
Creams	90 days	One 30 mL bottle holds 120 clicks; written for 90 and the remainder discarded

Billing stays quarterly - the charge codes and the price are unchanged. Quarterly for money, 12 weeks for the visit.

Who pays the pharmacy

The pharmacy note depends on WHERE it goes, not on whether the product is commercial or compounded.

- Partner pharmacy (Premier or Belmar): KORB pays the pharmacy, the patient pays KORB. The note says Bill to office/ship to patient. True for a commercial patch filled by Premier just as much as for a compound.
- Local pharmacy: the patient pays the pharmacy as a cash customer. The note must NOT say bill to office.
- Only commercially available products can go to a local pharmacy - in this programme, the estradiol patches. Every cream and every compounded capsule goes to a partner pharmacy, full stop.

Estrogen, estradiol, estriol - not the same word

ESTROGEN is the class. ESTRADIOL is one hormone in that class. The three human estrogens are estrone (E1), estradiol (E2) and estriol (E3).

ESTRADIOL is the most potent and the dominant estrogen through the reproductive years. It is what estrogen therapy almost always means at KORB, and it is what the patches and the single-hormone creams contain.

ESTRIOL is the weakest of the three, roughly a tenth as potent at the receptor, and predominates in pregnancy. It appears at KORB in exactly one place: the four-hormone capsule, at 0.45 mg alongside 0.45 mg of estradiol.

So Estriol/Estradiol on the Belmar capsule is NOT a typo and not a duplicate. They are two different hormones and the capsule contains both. A combination of estriol and estradiol is sometimes called Bi-Est. The near-identical spelling is the only thing they share.

On a prescription the two are not interchangeable words. Write the hormone, not the class.

Start with single hormones, combine later

A combination product is one prescription, so every component moves together. If a patient on the four-hormone capsule needs more progesterone, there is no way to give her more progesterone - only more capsule, which also raises her testosterone, her estradiol and her estriol.

So START on single-hormone products while dose-finding. Adjust one thing at a time, the same discipline as every other KORB programme, and let symptoms and labs settle.

ONCE SHE IS STABLE, moving to a combination is reasonable and is often better for adherence - one cream or one capsule instead of three. The trade-off is that the next adjustment means unpicking it again.

Convenience is the reason to combine, and it is a good reason - but only after the doses are settled. A combination started on day one is a plan that cannot be tuned.

The two gates - check these before you send

Both of these make a prescription wrong rather than suboptimal.

- Unopposed estrogen in a patient with a uterus causes endometrial hyperplasia and can lead to cancer. Add progesterone before sending.
- A Medical Necessity Statement is required for any compounded item. It is wider than the local-pharmacy block: it applies whenever a compounded product is dispensed, whichever partner pharmacy fills it.

Estradiol alone is first-line for a patient without a uterus. Progesterone is not required to protect an endometrium that is not there.

Which pharmacy fills for your patient

Premier is the default wherever Premier is licensed. Belmar covers every remaining state. Both lists are computed from the shared pharmacy layer, not maintained here.

Premier Pharmacy 38 states

Arizona · Colorado · Connecticut · District of Columbia · Delaware · Florida · Georgia · Illinois · Kansas · Kentucky · Louisiana · Maryland · Maine · Michigan · Missouri · Mississippi · Montana · North Carolina · North Dakota · Nebraska · New Jersey · New Mexico · Nevada · New York · Ohio · Oklahoma · Oregon · Pennsylvania · Rhode Island · South Dakota · Tennessee · Texas · Utah · Virginia · Vermont · Wisconsin · West Virginia · Wyoming

Belmar Pharmacy 13 states

Alaska · Alabama · Arkansas · California · Hawaii · Iowa · Idaho · Indiana · Massachusetts · Minnesota · New Hampshire · South Carolina · Washington

Belmar has several locations. KORB uses the ARIZONA address. 12012 N 111th Ave, Youngtown, AZ 85363-1339. Confirm that is the Belmar selected in Tebra before sending. Another Belmar location will be the wrong one.

Testosterone - two states only

Texas and California only, because those are the only states where a KORB provider holds the required DEA registration.

- Texas (TX) - may only be prescribed by Don Stevenson, PA-C.
- California (CA) - may only be prescribed by Larisa Hammond, NP or LaTonya King, DNP.
- Outside those two states, remove testosterone from the plan. Every other hormone in the programme is still available.
- Testosterone comes from Belmar, except in Texas where it comes from Premier - which follows from the routing rule rather than being a separate one.

Testosterone may only be prescribed in a state by the provider named for it. If that is not you, do not send the prescription.

What each pharmacy stocks

The strengths are NOT the same at both pharmacies, and that is the commonest way a women's health prescription goes wrong: a patient moves between pharmacies and the old strength is carried across.

	Premier	Belmar
Estradiol cream	0.2% (0.5 mg/click); 0.6% (1.5 mg/click)	0.2% (0.5 mg/click); 0.4% (1 mg/click); 0.8% (2 mg/click)
Estradiol patch	Yes	No
Progesterone	Both commercial and compounded. State which you want.	Compounded only.
Testosterone	Texas only, and restricted to named prescribers.	Every state except Texas - but see the testosterone restriction: in practice that means California only.

Tebra lists ONCE-weekly and TWICE-weekly transdermal patches. Prefer the ONCE-weekly if the patient can get it - fewer applications and it is the presentation these entries are written for.

Once-weekly patches come in a BOX OF 4. So the quantity is 3 BOXES, not 3 patches: $3 \times 4 = 12$ patches, one a week for 12 weeks. The pharmacy note spells that out so the pharmacy tells us if their pack size differs.

Only 0.2% (0.5 mg/click) exists at both. Every other estradiol cream strength changes when the pharmacy changes, so the prescription has to change with it.

Writing a cream prescription

Percent to mg per mL: multiply by 10. One click is 0.25 mL, so one click delivers a quarter of the mg/mL figure.

Write the sig in BOTH mg and clicks. The pharmacies ask for it that way.

- 2% testosterone = 20 mg/mL, so 1 click = 5 mg.
- 0.6% estradiol = 6 mg/mL, so 1 click = 1.5 mg.
- A 30 mL tube is 120 clicks at 0.25 mL each.
- Apply to hairless, thin skin - inner thigh, labia, back of the upper arm, forearm. Let it dry before contact with another person, to prevent transfer. Wash hands after applying. Patches go on the lower abdomen or upper buttock.

Pricing and charge codes

Every plan below is a VISIT plus a 90-DAY SUPPLY plus SHIPPING. Only what differs is in the table.

Tiered by how many HORMONES the plan contains, not how many prescriptions are written. One product can carry more than one: a progesterone and estradiol cream is two hormones in a single tube, and bills as two.

Plan	Price	Charge code
Local pharmacy - 90-day supply, patient pays the pharmacy	\$79	WMNHlthINS
1 hormone from a partner pharmacy	\$220	WMNHlth1
2 hormones from a partner pharmacy	\$250	WMNHlth2
3 hormones from a partner pharmacy	\$299	WMNHlth3

The \$79 covers the VISIT and the prescription. The patient pays their own pharmacy for the medication, as a cash customer. KORB does not bill insurance for it.

If a patient chooses to run the prescription through their insurance, that is their decision and their paperwork. KORB does NOT complete prior authorisations, medical necessity letters, appeals or any other insurance documentation for these prescriptions. None of that is included in the \$79, and it is not available separately - refer the patient to their primary care provider for it.

ONE 90-day supply, sent and billed at the same time. Not monthly, and not two 60-day fills billed as one quarter. No mid-cycle visits for dose adjustments either - a change between quarters is another visit and another charge, not a free follow-up.

If the patient needs to be seen between 90-day supplies, that visit is charged at \$79 (WMNHlthINS) like any other visit. Do not send a short supply and bill it against the next quarter - that is where charges get missed, and it is the commonest billing error on this programme.

Four hormones bill at the three-hormone tier, \$299 / WMNHlth3. There is no fourth tier.

Pricing with labs included

Same visit and 90-day supply as above, with a lab panel added. Labs on their own, without a prescription, are in the Labs section.

Hormones	Labs included	Price	Charge code
1 hormone	Basic Hormones (saliva)	\$339	WMNHlth1Bsc
1 hormone	Complete Hormones (saliva and blood spot)	\$469	WMNHlth1Cmp
2 hormones	Basic Hormones (saliva)	\$369	WMNHlth2Bsc
2 hormones	Complete Hormones (saliva and blood spot)	\$499	WMNHlth2Cmp
3 hormones	Basic Hormones (saliva)	\$418	WMNHlth3Bsc

Hormones	Labs included	Price	Charge code
3 hormones	Complete Hormones (saliva and blood spot)	\$548	WMNHlth3Cmp

Prescriptions and dosing are quarterly - one visit, one 90-day supply.

Labs on their own

Without a prescription. Collected at home through Ayumetrix. At-home collection. This is the only KORB programme that does not order through the Quest integration in Tebra.

Panel	Collection	Price	Charge code
Women Basic Hormones	Saliva	\$119	LABHlthBsc
Women Health Complete Hormones	Saliva and blood spot	\$249	LABHlthCmp

Saliva measures the bioavailable, non-protein-bound fraction that can diffuse into tissue, so it reflects what a TOPICAL hormone actually delivers. Serum does not rise meaningfully after topical dosing, so a normal serum level on a patient using a cream tells you very little.

Tebra entries

Twenty-eight entries. Every one selects its drug from the Tebra drop-down; the compounded items then carry their real formulation in the pharmacy note.

Estrogen

Premier Pharmacy · Tebra Standard prescription · Estrogen (0.025 mg Patch)		84-DAY SUPPLY
Drug	estradiol 0.025 mg/24 hr weekly transdermal patch Select this from the Tebra drop-down. Do not copy and paste.	
Name	Estrogen 0.025 mg Patch	
Allow Substitution	Yes - select Allow Substitution	
Quantity	3	
Unit	BX	
Refill	0	
Days Supply	84	
Patient Instructions	Apply 1 patch to the lower abdomen or buttocks weekly. Replace the same days each week. Avoid breasts/irritated skin. Rotate sites.	
Pharmacy Instructions - PARTNER pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Bill to office/ship to patient	
Pharmacy Instructions - LOCAL pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Patient pays the pharmacy.	

These are an 84-DAY supply, not 90. Twelve weekly patches is twelve weeks. Book the follow-up at 12 weeks. Corrected 2026-09-16 after Don raised it - the entries had read 90 since the source document.

Premier Pharmacy · Tebra Standard prescription · Estrogen (0.0375 mg Patch)		84-DAY SUPPLY
Drug	estradiol 0.0375 mg/24 hr weekly transdermal patch Select this from the Tebra drop-down. Do not copy and paste.	
Name	Estrogen 0.0375 mg Patch	
Allow Substitution	Yes - select Allow Substitution	
Quantity	3	
Unit	BX	
Refill	0	
Days Supply	84	
Patient Instructions	Apply 1 patch to the lower abdomen or buttocks weekly. Replace the same days each week. Avoid breasts/irritated skin. Rotate sites.	
Pharmacy Instructions - PARTNER pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Bill to office/ship to patient	
Pharmacy Instructions - LOCAL pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Patient pays the pharmacy.	

These are an 84-DAY supply, not 90. Twelve weekly patches is twelve weeks. Book the follow-up at 12 weeks. Corrected 2026-09-16 after Don raised it - the entries had read 90 since the source document.

Premier Pharmacy · Tebra Standard prescription · Estrogen (0.05 mg Patch)		84-DAY SUPPLY
Drug	estradiol 0.05 mg/24 hr weekly transdermal patch Select this from the Tebra drop-down. Do not copy and paste.	
Name	Estrogen 0.05 mg Patch	
Allow Substitution	Yes - select Allow Substitution	
Quantity	3	
Unit	BX	
Refill	0	
Days Supply	84	
Patient Instructions	Apply 1 patch to the lower abdomen or buttocks weekly. Replace the same days each week. Avoid breasts/irritated skin. Rotate sites.	
Pharmacy Instructions - PARTNER pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Bill to office/ship to patient	
Pharmacy Instructions - LOCAL pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Patient pays the pharmacy.	

These are an 84-DAY supply, not 90. Twelve weekly patches is twelve weeks. Book the follow-up at 12 weeks. Corrected 2026-09-16 after Don raised it - the entries had read 90 since the source document.

Premier Pharmacy · Tebra Standard prescription · Estrogen (0.075 mg Patch)		84-DAY SUPPLY
Drug	estradiol 0.075 mg/24 hr weekly transdermal patch Select this from the Tebra drop-down. Do not copy and paste.	
Name	Estrogen 0.075 mg Patch	
Allow Substitution	Yes - select Allow Substitution	
Quantity	3	
Unit	BX	
Refill	0	
Days Supply	84	
Patient Instructions	Apply 1 patch to the lower abdomen or buttocks weekly. Replace the same days each week. Avoid breasts/irritated skin. Rotate sites.	
Pharmacy Instructions - PARTNER pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Bill to office/ship to patient	
Pharmacy Instructions - LOCAL pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Patient pays the pharmacy.	

These are an 84-DAY supply, not 90. Twelve weekly patches is twelve weeks. Book the follow-up at 12 weeks. Corrected 2026-09-16 after Don raised it - the entries had read 90 since the source document.

Premier Pharmacy · Tebra Standard prescription · Estrogen (0.1 mg Patch)		84-DAY SUPPLY
Drug	estradiol 0.1 mg/24 hr weekly transdermal patch Select this from the Tebra drop-down. Do not copy and paste.	
Name	Estrogen 0.1 mg Patch	
Allow Substitution	Yes - select Allow Substitution	
Quantity	3	
Unit	BX	
Refill	0	
Days Supply	84	
Patient Instructions	Apply 1 patch to the lower abdomen or buttocks weekly. Replace the same days each week. Avoid breasts/irritated skin. Rotate sites.	
Pharmacy Instructions - PARTNER pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Bill to office/ship to patient	
Pharmacy Instructions - LOCAL pharmacy	3 boxes of 4 patches = 12 patches, one weekly for 12 weeks. Patient pays the pharmacy.	

These are an 84-DAY supply, not 90. Twelve weekly patches is twelve weeks. Book the follow-up at 12 weeks. Corrected 2026-09-16 after Don raised it - the entries had read 90 since the source document.

Estrogen

Belmar Pharmacy · Compounded Drug Favorite Entry · Estrogen (0.2% Cream)		90-DAY SUPPLY
Drug	Estradiol 0.2%, 2 mg/ml cream	
Name	Belmar Estradiol Cream 0.2%	
Allow Substitution	Yes - select Allow Substitution	
Quantity	1	
Unit	bottle	
Refill	0	
Days Supply	90	
Patient Instructions	Apply 1 click/0.25ml daily to hairless skin. Let dry. Avoid contact. Wash hands after.	
Reason for Compounding	Strength not commercial	
Pharmacy Instructions	Medical Necessity Required, QTY 30 ml, Bill to office/ship to patient, Allergies:	

Premier Pharmacy · Compounded Drug Favorite Entry · Estrogen (0.2% Cream)		90-DAY SUPPLY
Drug	Estradiol 0.2%, 2 mg/ml cream	
Name	Estradiol Cream 0.2%	
Allow Substitution	Yes - select Allow Substitution	
Quantity	1	
Unit	bottle	
Refill	0	
Days Supply	90	
Patient Instructions	Apply (1 click/0.25ml/0.5 mg) daily to hairless skin. Let dry. Avoid contact. Wash hands after.	
Reason for Compounding	Strength not commercial	
Pharmacy Instructions	QTY 30 ml, Bill office/ship to patient	

Belmar Pharmacy · Compounded Drug Favorite Entry · Estrogen (0.4% Cream)
90-DAY SUPPLY

Drug	Estradiol 0.4%, 4 mg/ml cream
Name	Belmar Estradiol Cream 0.4%
Allow Substitution	Yes - select Allow Substitution
Quantity	1
Unit	bottle
Refill	0
Days Supply	90
Patient Instructions	Apply 1 click/0.25ml daily to hairless skin. Let dry. Avoid contact. Wash hands after.
Reason for Compounding	Strength not commercial
Pharmacy Instructions	QTY 30 ml, Bill office/ship patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Estrogen (0.6% Cream)
90-DAY SUPPLY

Drug	Estradiol 0.6%, 6 mg/ml cream
Name	Estradiol Cream 0.6%
Allow Substitution	Yes - select Allow Substitution
Quantity	1
Unit	bottle
Refill	0
Days Supply	90
Patient Instructions	Apply (1 click/0.25 ml/1.5 mg) daily to hairless skin. Let dry. Avoid contact. Wash hands after.
Reason for Compounding	Strength not commercial
Pharmacy Instructions	QTY 30 ml, Bill office/ship to patient

Belmar Pharmacy · Compounded Drug Favorite Entry · Estrogen (0.8% Cream)		90-DAY SUPPLY
Drug	Estradiol 0.8%, 8 mg/ml cream	
Name	Belmar Estradiol Cream 0.8%	
Allow Substitution	Yes - select Allow Substitution	
Quantity	1	
Unit	bottle	
Refill	0	
Days Supply	90	
Patient Instructions	Apply 1 click/0.25ml daily to hairless skin. Let dry. Avoid contact. Wash hands after.	
Reason for Compounding	Strength not commercial	
Pharmacy Instructions	Medical Necessity Required, QTY 30 ml, Bill office/ship to patient, Allergies:	

Progesterone

Belmar Pharmacy · Compounded Drug Favorite Entry · Progesterone (100 mg Capsule)		90-DAY SUPPLY
Drug	Progesterone (clear) MCC 100 mg capsule	
Name	Belmar Progesterone MCC 100 mg NO cycles	
Allow Substitution	Yes - select Allow Substitution	
Quantity	90	
Unit	capsule	
Refill	0	
Days Supply	90	
Patient Instructions	Take 1 cap PO QHS	
Reason for Compounding	Peanut-free base	
Pharmacy Instructions	Bill office/ship to patient Allergies:	

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone (100 mg Capsule)

90-DAY SUPPLY

Drug	Progesterone SR 100 mg capsule
Name	Progesterone SR 100 mg NO cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	capsule
Refill	0
Days Supply	90
Patient Instructions	Take 1 cap PO QHS
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill to office/ship to patient

Belmar Pharmacy · Compounded Drug Favorite Entry · Progesterone (200 mg Capsule)

90-DAY SUPPLY

Drug	Progesterone (clear) MCC 200 mg capsule
Name	Belmar Progesterone MCC 200 mg NO cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	capsule
Refill	0
Days Supply	90
Patient Instructions	Take 1 cap PO QHS
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill office/ship to patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone (200 mg Capsule)

90-DAY SUPPLY

Drug	Progesterone SR 200 mg capsule
Name	Progesterone SR 200 mg NO cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	capsule
Refill	0
Days Supply	90
Patient Instructions	Take 1 cap PO QHS
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill to office/ship to patient

Belmar Pharmacy · Compounded Drug Favorite Entry · Progesterone (300 mg Capsule)

90-DAY SUPPLY

Drug	Progesterone (clear) MCC 300 mg capsule
Name	Belmar Progesterone MCC 300 mg NO cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	capsule
Refill	0
Days Supply	90
Patient Instructions	Take 1 cap PO QHS
Reason for Compounding	Strength not commercial
Pharmacy Instructions	Bill office/ship to patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone (300 mg Capsule)

90-DAY SUPPLY

Drug	Progesterone SR 300 mg capsule
Name	Progesterone SR 300 mg NO cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	capsule
Refill	0
Days Supply	90
Patient Instructions	Take 1 cap PO QHS
Reason for Compounding	Strength not commercial
Pharmacy Instructions	Bill to office/ship to patient

Belmar Pharmacy · Compounded Drug Favorite Entry · Progesterone (100 mg Capsule)

84-DAY SUPPLY

Drug	Progesterone (clear) MCC 100 mg capsule
Name	Belmar Progesterone MCC 100 mg with cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	63
Unit	capsule
Refill	0
Days Supply	84
Patient Instructions	Take 1 cap PO QHS, cycle 21 days on / 7 days off
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill office/ship to patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone (100 mg Capsule)

84-DAY SUPPLY

Drug	Progesterone SR 100 mg capsule
Name	Progesterone SR 100 mg with cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	63
Unit	capsule
Refill	0
Days Supply	84
Patient Instructions	Take 1 cap PO QHS, cycle 21 days on / 7 days off
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill to office/ship to patient

Belmar Pharmacy · Compounded Drug Favorite Entry · Progesterone (200 mg Capsule)

84-DAY SUPPLY

Drug	Progesterone (clear) MCC 200 mg capsule
Name	Belmar Progesterone MCC 200 mg with cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	63
Unit	capsule
Refill	0
Days Supply	84
Patient Instructions	Take 1 cap PO QHS, cycle 21 days on / 7 days off
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill office/ship to patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone (200 mg Capsule)

84-DAY SUPPLY

Drug	Progesterone SR 200 mg capsule
Name	Progesterone SR 200 mg with cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	63
Unit	capsule
Refill	0
Days Supply	84
Patient Instructions	Take 1 cap PO QHS, cycle 21 days on / 7 days off
Reason for Compounding	Peanut-free base
Pharmacy Instructions	Bill to office/ship to patient

Belmar Pharmacy · Compounded Drug Favorite Entry · Progesterone (300 mg Capsule)

84-DAY SUPPLY

Drug	Progesterone (clear) MCC 300 mg capsule
Name	Belmar Progesterone MCC 300 mg with cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	63
Unit	capsule
Refill	0
Days Supply	84
Patient Instructions	Take 1 cap PO QHS, cycle 21 days on / 7 days off
Reason for Compounding	Strength not commercial
Pharmacy Instructions	Bill office/ship to patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone (300 mg Capsule)

84-DAY SUPPLY

Drug	Progesterone SR 300 mg capsule
Name	Progesterone SR 300 mg with cycles
Allow Substitution	Yes - select Allow Substitution
Quantity	63
Unit	capsule
Refill	0
Days Supply	84
Patient Instructions	Take 1 cap PO QHS, cycle 21 days on / 7 days off
Reason for Compounding	Strength not commercial
Pharmacy Instructions	Bill to office/ship to patient

Testosterone

Belmar Pharmacy · Compounded Drug Favorite Entry · Testosterone (2% Cream)

90-DAY SUPPLY

Drug	Testosterone 2%, 20 mg/ml cream
Name	Belmar Testosterone 2% Cream
Allow Substitution	Yes - select Allow Substitution
Quantity	1
Unit	bottle
Refill	0
Days Supply	90
Patient Instructions	Apply 1 click 0.25 ml daily to hairless skin. Let dry. Avoid contact. Wash hands after.
Reason for Compounding	No female-dose product
Pharmacy Instructions	QTY 30 ml, Bill office/ship patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Testosterone (2% Cream)

90-DAY SUPPLY

Drug	Testosterone 2%, 20 mg/ml cream
Name	Testosterone Cream Premier Pharmacy
Allow Substitution	Yes - select Allow Substitution
Quantity	1
Unit	bottle
Refill	0
Days Supply	90
Patient Instructions	Apply (1 click/0.25 ml/5 mg) daily to hairless skin. Let dry. Avoid contact. Wash hands after
Reason for Compounding	No female-dose product
Pharmacy Instructions	QTY 23 ml, Bill to office/ship to patient

Estradiol/Testosterone

Belmar Pharmacy · Compounded Drug Favorite Entry · Estradiol/Testosterone (Cream)

28-DAY SUPPLY

Drug	Estradiol/Testosterone 4/20 mg/ml cream
Name	Belmar Estradiol/Testosterone (Topical Cream)
Allow Substitution	Yes - select Allow Substitution
Quantity	1
Unit	bottle
Refill	2
Days Supply	28
Patient Instructions	Apply one click/0.25 ml daily to hairless skin. Let dry. Avoid contact. Wash hands after.
Reason for Compounding	Multi-active combination
Pharmacy Instructions	QTY 30 ml, Bill office/ship patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Testosterone + Estradiol (Cream)

90-DAY SUPPLY

Drug	Testosterone 2% & Estradiol 0.4%, (20 mg/4 mg)/ml cream
Name	Testosterone + Estradiol (Cream) Premier Pharmacy
Allow Substitution	Yes - select Allow Substitution
Quantity	1
Unit	bottle
Refill	0
Days Supply	90
Patient Instructions	Apply (1 click/0.25 ml/5 mg/0.1 mg) daily to hairless skin. Let dry. Avoid contact. Wash hands after
Reason for Compounding	Multi-active combination
Pharmacy Instructions	QTY 23 ml, Bill to office/ship to patient

Estriol/ Estradiol / Progesterone / Testosterone

Belmar Pharmacy · Compounded Drug Favorite Entry · Estriol/ Estradiol / Progesterone / Testosterone (Capsule)

90-DAY SUPPLY

Drug	Estriol/Estradiol/Progesterone/Testosterone 0.45/0.45/100/4 mg cap
Name	Belmar Estriol/Estradiol/Progesterone/Testosterone (capsule)
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	capsule
Refill	0
Days Supply	90
Patient Instructions	Take 1 capsule PO daily.
Reason for Compounding	Multi-active combination
Pharmacy Instructions	Bill office/ship patient Allergies:

Premier Pharmacy · Compounded Drug Favorite Entry · Progesterone / Testosterone / Estriol/ Estradiol (Tablet)

90-DAY SUPPLY

Drug	Progesterone 100 mg/Testosterone 4 mg/ Estriol 0.45 mg/Estradiol 0.45 mg ODT tablet
Name	Progesterone / Testosterone / Estriol / Estradiol (SL ODT Tab)
Allow Substitution	Yes - select Allow Substitution
Quantity	90
Unit	tablet
Refill	0
Days Supply	90
Patient Instructions	Dissolve 1 tablet under the tongue daily
Reason for Compounding	Multi-active combination
Pharmacy Instructions	Bill office/ship patient

What the panels measure, and what each result means

Basic panel (saliva): Estradiol (E2), Testosterone (T), Progesterone (Pg), DHEA, Cortisol x 1.

Complete panel adds a four-point cortisol and a blood spot for TSH, Free T3, Free T4, Thyroid peroxidase (TPO) antibodies.

Marker	What it means
Estradiol (E2)	Acts mainly as a growth hormone for the female reproductive structures, and works with progesterone through the cycle. Low estrogen can cause low libido; too much causes the symptoms of estrogen dominance. It also maintains bone growth, improves coronary blood flow and is neuroprotective. Estrogens contribute to breast cancer risk and to non-cancerous conditions such as endometriosis and uterine fibroids.
Testosterone (T)	Maintains bone strength, muscle mass and energy, and in women drives libido. Menopause causes a significant decline. Low levels can produce hair loss, reduced muscle mass, hot flashes, depression and increased breast size. High levels are linked with aggression, acne, liver disease and heart muscle damage.

Marker	What it means
Progesterone (Pg)	Produced mainly by the corpus luteum after ovulation, with a smaller adrenal contribution. It opposes estrogen at the endometrium, which is why it is what protects the uterine lining. Through its metabolite allopregnanolone it also has a calming effect and supports sleep, which is why it is dosed at night. Levels fall through perimenopause and are low after menopause. Low progesterone RELATIVE to estrogen gives the picture usually called estrogen dominance - heavy or irregular bleeding, breast tenderness, fluid retention, irritability and poor sleep. On saliva testing in a woman using a topical, remember the result reflects tissue delivery rather than a serum level.
DHEA	Made by the adrenal glands and a precursor to both testosterone and the estrogens; also a neurohormone made in small amounts in the brain. Improves energy, mood, memory, libido and immune function. In women it helps balance the other hormones, and low levels can cause weight gain, depression, fatigue and low libido.
Cortisol	The stress hormone, and also central to glucose metabolism. Imbalance produces irritability, fatigue, depression, foggy thinking, weight gain and bone loss. The Complete panel measures it four times across a day rather than once, because the shape of the curve is the finding.
TSH	Elevated in primary hypothyroidism, low in primary hyperthyroidism. Distinguishes primary thyroid disease from secondary (pituitary) and tertiary (hypothalamic). An abnormal TSH with a normal free T4 is subclinical hypo- or hyperthyroidism.
Free T3	Only about 0.3% of T3 is unbound, and the free fraction is the active one. Usually raised alongside T4 in hyperthyroidism; in a small subset - T3 toxicosis - only T3 is raised.
Free T4	A small fraction of total thyroxine and the metabolically active part. Raised causes hyperthyroidism, low causes hypothyroidism.
TPO antibodies	Thyroid disorders are often autoimmune. In subclinical hypothyroidism, TPO antibodies predict a higher rate of progression to overt disease - 4.3% per year against 2.1% without them - and flag risk of other autoimmune disease such as adrenal insufficiency and type 1 diabetes.

Ayumetrix developed these tests and determined their performance characteristics. They have not been cleared or approved by the FDA. The lab is CLIA-regulated as qualified for high-complexity testing. Results are informational and do not themselves provide a diagnosis.

Side effects to counsel on

Hormone	What to tell her
Estrogens - estradiol and estriol	Headache, nausea, bloating, mood swings, breast swelling and tenderness, change in vaginal bleeding. Increased risk of blood clots, breast cancer, and endometrial cancer - the last of which is reduced by adding progesterone.

Hormone	What to tell her
Progesterone	Headache, mood swings, breast tenderness, change in vaginal bleeding.
Testosterone	Acne, facial and body hair growth, voice deepening, weight gain. Voice deepening does not reverse - counsel before starting, not after.

Contraindications - do not prescribe

Hormone therapy is not recommended for women with any of the following. Refer to the PCP or OBGYN. Do not prescribe.

- History of breast cancer, endometrial cancer, or any estrogen-sensitive cancer
- History of venous thromboembolism or a clotting disorder
- History of stroke or heart attack, or uncontrolled hypertension
- Active liver disease
- Unexplained vaginal bleeding

Questions and escalation

- Pharmacy or shipping issues — Operations
- Charge codes and billing — Clinical Operations
- Clinical protocol questions — Clinical Operations
- Corrections to this document — Clinical Operations

Route everything through Clinical Operations. Do not contact the clinical sign-off physician directly; Clinical Operations escalates when it needs to.

Do not improvise

- Do not alter quantity, refill or days supply
- Do not paraphrase patient instructions
- Do not cross a gender-specific product
- Do not quote pricing to a patient without confirming with Operations

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